

HIGHLIGHTS OF PRESCRIBING INFORMATION
These highlights do not include all the information needed to use YESINTEK safely and effectively. See full prescribing information for YESINTEK.

YESINTEK™ (ustekinumab-kfce) injection, for subcutaneous or intravenous use
Initial U.S. Approval: 2024

YESINTEK™ (ustekinumab-kfce) is biosimilar* to STELARA® (ustekinumab).

INDICATIONS AND USAGE

YESINTEK is a human interleukin-12 and -23 antagonist indicated for the treatment of:

Adult patients with:

- moderate to severe plaque psoriasis (PsO) who are candidates for phototherapy or systemic therapy. (1.1)
- active psoriatic arthritis (PsA). (1.2)
- moderately to severely active Crohn's disease (CD). (1.3)
- moderately to severely active ulcerative colitis. (1.4)

Pediatric patients 6 years and older with:

- moderate to severe plaque psoriasis, who are candidates for phototherapy or systemic therapy. (1.1)
- active psoriatic arthritis (PsA). (1.2)

DOSAGE AND ADMINISTRATION

Psoriasis Adult Subcutaneous Recommended Dosage (2.1):

Weight Range (kilograms)	Recommended Dosage
less than or equal to 100 kg	45 mg administered subcutaneously initially and 4 weeks later, followed by 45 mg administered subcutaneously every 12 weeks
greater than 100 kg	90 mg administered subcutaneously initially and 4 weeks later, followed by 90 mg administered subcutaneously every 12 weeks

Psoriasis Pediatric Patients (6 to 17 years old) Subcutaneous Recommended Dosage (2.1): Weight-based dosing is recommended at the initial dose, 4 weeks later, then every 12 weeks thereafter.

Weight Range (kilograms)	Dose
less than 60 kg	0.75 mg/kg
60 kg to 100 kg	45 mg
greater than 100 kg	90 mg

Psoriatic Arthritis Adult Subcutaneous Recommended Dosage (2.2):

- The recommended dosage is 45 mg administered subcutaneously initially and 4 weeks later, followed by 45 mg administered subcutaneously every 12 weeks.
- For patients with co-existent moderate-to-severe plaque psoriasis weighing greater than 100 kg, the recommended dosage is 90 mg administered subcutaneously initially and 4 weeks later, followed by 90 mg administered subcutaneously every 12 weeks.

Psoriatic Arthritis Pediatric (6 to 17 years old) Subcutaneous Recommended Dosage (2.2): Weight-based dosing is recommended at the initial dose, 4 weeks later, then every 12 weeks thereafter.

Weight Range (kilograms)	Dose
less than 60 kg	0.75 mg/kg
60 kg or more	45 mg
greater than 100 kg with co-existent moderate-to-severe plaque psoriasis	90 mg

Crohn's Disease and Ulcerative Colitis Initial Adult Intravenous Recommended Dosage (2.3): A single intravenous infusion using weight-based dosing:

Weight Range (kilograms)	Dose
up to 55 kg	260 mg (2 vials)
greater than 55 kg to 85 kg	390 mg (3 vials)
greater than 85 kg	520 mg (4 vials)

Crohn's Disease and Ulcerative Colitis Maintenance Adult Subcutaneous Recommended Dosage (2.3): A subcutaneous 90 mg dose 8 weeks after the initial intravenous dose, then every 8 weeks thereafter.

DOSAGE FORMS AND STRENGTHS

Subcutaneous Injection (3)

- Injection: 45 mg/0.5 mL or 90 mg/mL solution in a single-dose prefilled syringe
- Injection: 45 mg/0.5 mL solution in a single-dose vial

Intravenous Infusion (3)

- Injection: 130 mg/26 mL (5 mg/mL) solution in a single-dose vial (3)

CONTRAINDICATIONS

Clinically significant hypersensitivity to ustekinumab products or to any of the excipients in YESINTEK. (4)

WARNINGS AND PRECAUTIONS

- Infections: Serious infections have occurred. Avoid starting YESINTEK during any clinically important active infection. If a serious infection or clinically significant infection develops, discontinue YESINTEK until the infection resolves. (5.1)
- Theoretical Risk for Particular Infections: Serious infections from mycobacteria, salmonella and Bacillus Calmette-Guerin (BCG) vaccinations have been reported in patients genetically deficient in IL-12/IL-23. Consider diagnostic tests for these infections as dictated by clinical circumstances. (5.2).
- Tuberculosis (TB): Evaluate patients for TB prior to initiating treatment with YESINTEK. Initiate treatment of latent TB before administering YESINTEK. (5.3).
- Malignancies: Ustekinumab products may increase risk of malignancy. The safety of ustekinumab products in patients with a history of or a known malignancy has not been evaluated. (5.4)
- Hypersensitivity Reactions: If an anaphylactic or other clinically significant hypersensitivity reaction occurs, institute appropriate therapy and discontinue YESINTEK. (5.5)
- Posterior Reversible Encephalopathy Syndrome (PRES): If PRES is suspected, treat promptly and discontinue YESINTEK. (5.6)
- Immunizations: Avoid use of live vaccines in patients during treatment with YESINTEK. (5.7)
- Noninfectious Pneumonia: Cases of interstitial pneumonia, eosinophilic pneumonia and cryptogenic organizing pneumonia have been reported during post-approval use of ustekinumab products. If diagnosis is confirmed, discontinue YESINTEK and institute appropriate treatment. (5.8)

ADVERSE REACTIONS

Most common adverse reactions are:

- Psoriasis (≥3%): nasopharyngitis, upper respiratory tract infection, headache, and fatigue. (6.1)
- Crohn's Disease, induction (≥3%): vomiting. (6.1)
- Crohn's Disease, maintenance (≥3%): nasopharyngitis, injection site erythema, vulvovaginal candidiasis/mycotic infection, bronchitis, pruritus, urinary tract infection, and sinusitis. (6.1)
- Ulcerative colitis, induction (≥3%): nasopharyngitis (6.1)
- Ulcerative colitis, maintenance (≥3%): nasopharyngitis, headache, abdominal pain, influenza, fever, diarrhea, sinusitis, fatigue, and nausea (6.1)

To report SUSPECTED ADVERSE REACTIONS, contact Biocon

Biologics at 1-833-986-1468 or FDA at 1-800-FDA-1088 or www.fda.gov/medwatch.

See 17 for PATIENT COUNSELING INFORMATION and Medication Guide.

*Biosimilar means that the biological product is approved based on data demonstrating that it is highly similar to an FDA-approved biological product, known as a reference product, and that there are no clinically meaningful differences between the biosimilar product and the reference product. Biosimilarity of YESINTEK has been demonstrated for the condition(s) of use (e.g., indication(s), dosing regimen(s), strength(s), dosage form(s), and route(s) of administration described in its Full Prescribing Information.

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